

# Multimodal Neonatal Pain Classification Using EEG, ECG, and Respiration Signals with Subject-Level Validation

Arpit Pandey\*, Alka Verma\*, Vimal Mishra \*, Yogendra Kumar Prajapati <sup>†</sup>, *Senior Member, IEEE*

\*Electronics Engineering, Institute of Engineering and Rural Technology, Prayagraj, India

<sup>†</sup>ECED, Motilal Nehru National Institute of Technology Allahabad, Prayagraj, India

**Abstract**—Neonatal pain assessment is a critical clinical challenge, as preterm infants undergoing repeated procedural pain cannot verbally communicate their discomfort, and existing observational tools such as the Premature Infant Pain Profile (PIPP) are inherently subjective and unsuitable for continuous automated monitoring. This paper proposes a multimodal machine learning framework for objective neonatal pain classification using electroencephalography (EEG), electrocardiography (ECG), and respiration signals. A signal preprocessing pipeline was first validated on the PhysioNet Preterm Infant Cardiorespiratory Signals (PICS) dataset comprising 10 infants, where 26,208 ECG windows were successfully extracted and analysed. Multimodal pain classification was subsequently evaluated on the publicly available UCL neonatal pain dataset comprising 112 neonates and 308 recordings, using an XGBoost classifier with Leave-One-Subject-Out (LOSO) cross-validation. Two evaluation protocols were employed: a broad condition-based scheme and a stricter clinically-validated PIPP-confirmed subset. The proposed framework achieved a mean LOSO AUC of 0.849 on the condition-based cohort (53 evaluable subjects) and 0.929 on the PIPP-confirmed cohort (28 subjects), demonstrating competitive performance for automated multimodal neonatal pain assessment.

**Index Terms**—Neonatal pain assessment, Multimodal physiological signals, Electroencephalography (EEG), XGBoost, Leave-One-Subject-Out Cross-Validation, PIPP score.

## I. INTRODUCTION

### A. Background and Motivation

Neonates admitted to Neonatal Intensive Care Units (NICUs) are routinely subjected to numerous painful procedures, including heel lances, venipunctures, immunizations, and endotracheal suctioning. Repeated exposure to untreated or under-treated pain during this critical period of neurodevelopment has been linked to adverse long-term neurodevelopmental outcomes and altered pain perception. Because neonates lack the cognitive and verbal capacity to communicate their pain, clinicians must rely on alternative methods to assess pain accurately. Consequently, the development of reliable, objective pain assessment tools is of paramount importance to ensure adequate analgesia and mitigate the long-term consequences of neonatal pain.

### B. Current Clinical Assessment

In clinical practice, neonatal pain is predominantly assessed using observational scoring systems such as the Premature Infant Pain Profile (PIPP) [1], its revised version (PIPP-R) [2], the Neonatal Infant Pain Scale (NIPS) [3], and the Neonatal Facial Coding System (NFCS) [4]. What these scales do is combine what a nurse can see — things like whether the baby's brow is furrowed or whether they're crying — with numbers from a monitor, like heart rate and oxygen levels, adjusted for how premature the baby is. During practice, the collected score depends on who is doing the scoring. Two nurses who are trained, observing the same infant at the same moment routinely assign different totals, because the judgments involved are really difficult to standardize. A nurse also cannot score pain continuously — it happens before and after a procedure, with nothing in between. This gap, together with the problem of subjectivity, makes an automated alternative worth pursuing.

### C. Machine Learning Approaches

To address the limitations of manual scoring, researchers have increasingly turned to machine learning to decode neonatal pain from physiological signals. Early neurophysiological studies established that nociceptive stimuli elicit distinct cortical responses, such as nociceptive-specific event-related potentials (ERPs) in EEG. Building on this, feature-based machine learning methods have been applied to various physiological modalities, including EEG, ECG, and respiration. Recently, several studies have leveraged the publicly available UCL neonatal dataset to develop automated pain classifiers. For instance, Hamwi *et al.* (2025) [5] demonstrated strong classification performance (AUC 0.94, Accuracy 82%) using Logistic Regression restricted to the oldest gestational age group. Similarly, Rou  t *et al.* (2025) [6] achieved an AUC of 0.90 using Gradient Boosting on the full dataset. While these studies have established strong baselines, critical gaps remain. Most existing pipelines rely on single modalities or lack rigorous subject-independent validation. There is a pressing need for comprehensive frameworks that integrate multiple physiological modalities and are evaluated using strict Leave-One-Subject-Out (LOSO) cross-validation to ensure robust generalization across unseen patients.

\*Arpit Pandey: pandey07arpit@gmail.com.

<sup>†</sup>Yogendra Kumar Prajapati ORCID: 0000-0002-6752-5667.

Dr. Alka Verma ORCID: 0000-0001-7192-1972.

#### D. Contributions

In this work, the proposed framework is developed using two complementary datasets. The PICS dataset is employed to validate the ECG preprocessing and feature extraction methodology, whereas the publicly available UCL neonatal pain dataset is used for multimodal pain classification and subject-level evaluation.

- Development and validation of an ECG preprocessing and heart-rate variability (HRV) feature extraction pipeline using the PICS dataset.
- Construction of a multimodal neonatal pain classification framework integrating EEG spectral features, ERP features, ECG features, and respiration features extracted from the UCL dataset, while integrating EEG, ECG, and respiration for the first time in a fully multimodal framework evaluated with strict LOSO cross-validation.
- Subject-independent evaluation using Leave-One-Subject-Out (LOSO) cross-validation to assess generalization across unseen neonates.
- Competitive classification performance using XGBoost, achieving AUC values of 0.929 and 0.849, demonstrating performance comparable with recent state-of-the-art methods.

## II. RELATED WORK

### A. Clinical Pain Assessment in Neonates

The assessment of pain in neonates remains a challenging clinical task because newborn infants cannot verbally communicate pain. Consequently, observational pain assessment scales have become the clinical standard for evaluating procedural pain in neonatal intensive care units (NICUs). Among these, the Premature Infant Pain Profile (PIPP) [1] and its revised version (PIPP-R) are the most widely adopted. In practice, clinicians scoring pain on these scales are simultaneously watching the baby's face, reading the bedside monitor, and accounting for how premature the infant is — all folded into a single composite number that is meant to stand in for something that cannot be directly measured.

These tools have become clinical standard largely because nothing better existed, not because they are objectively reliable. When the same infant is scored by two different nurses, the results often disagree. And because scoring requires active observation, it simply cannot happen continuously — there are always gaps in the record during which a baby's pain state is unknown. Clinical assessment can also be influenced by gestational maturity, neurological development, and the administration of analgesics, motivating the search for objective physiological biomarkers capable of providing automated pain assessment.

### B. Physiological Biomarkers for Neonatal Pain

Advances in physiological monitoring have enabled the investigation of objective biomarkers associated with neonatal pain. Electroencephalography (EEG) provides direct measurements of cortical activity and has been shown to exhibit characteristic nociceptive event-related potentials (ERPs) following

painful stimulation. The pioneering work of Slater *et al.* [7] demonstrated that noxious procedures evoke reproducible cortical responses even in premature infants, establishing EEG as an important modality for neonatal pain assessment.

The body reacts more profoundly to pain than the face. When a needle touches a baby's heel, the speed of heart increases, the beat-to-beat timing becomes less regular, and the pattern of breathing changes sometimes subtly, sometimes dramatically. These autonomic signals can be measured through the ECG and a respiration belt. Pain results in irregularity in heart rate, heart-rate variability (HRV) [8], capture a dimension of the pain response that facial scoring completely misses. However, each individual modality is subject to motion artefacts, developmental variability, and environmental influences. Recent studies have therefore increased exploring multimodal approaches that integrate cortical and autonomic responses to obtain a more complete picture of neonatal pain.

### C. Machine Learning for Automated Neonatal Pain Assessment

Machine learning has become as a promising approach for translating multimodal physiological recordings into objective pain assessment tools. Initial works have primarily focused on handcrafted EEG features, including event-related potentials and spectral characteristics, while later work included autonomic features from ECG and respiration.

One dataset has become the most significant testing ground for this line of research — the UCL neonatal pain dataset, collected at University College London Hospital over nine years. The reason of its significance is not just its size (112 neonates, over 300 recordings) but the fact that brain activity, heart signals, and breathing were all captured simultaneously, synchronized on the point of the heel lance was performed, together PIPP scores assigned by trained clinicians.

Among recent studies, Hamwi *et al.* [5] achieved an AUC of 0.94 using Logistic Regression for older gestational-age infants. And, Roué *et al.* [6] achieved an AUC of 0.90 using a Gradient Boosting classifier trained on the complete multimodal dataset. These studies show the effectiveness of combining physiological biomarkers with machine learning, while also establishing strong performance baselines for the UCL dataset.

### D. Research Gap and Motivation

Although recent studies have demonstrated promising performance on neonatal pain classification, several important challenges remain. Existing approaches frequently focus on a limited subset of physiological features or employ evaluation strategies that may not adequately assess subject-level generalization. In addition, relatively few studies explicitly validate their signal preprocessing pipelines prior to large-scale model development.

The approach taken here tries to address both gaps at once. Before any pain classifier was trained, the signal processing pipeline was tested independently on a separate dataset — the PhysioNet PICS database — specifically to confirm

that the ECG cleaning and HRV extraction were producing sensible outputs on real neonatal recordings. Only after that validation was complete was the pipeline applied to the UCL pain data. ECG, and respiration is developed and evaluated on the UCL dataset using Leave-One-Subject-Out (LOSO) cross-validation. This way of subject-independent evaluation protocol clearly reflects the environment of clinical scenario in which the model is needed to generalize to previously unseen infants.

### III. MATERIALS AND METHODS

#### A. Datasets

1) *PICS Dataset (Pipeline Validation)*: The Pain in Infants Clinical Study (PICS) [9] dataset was used exclusively for validation of the proposed ECG preprocessing and feature extraction pipeline. The dataset consists of continuous ECG recordings collected from 10 infants during clinically required procedures. ECG signals were sampled at 500 Hz and processed using the NeuroKit2 [10] toolbox to perform signal cleaning, R-peak detection, and heart-rate variability (HRV)-analysis [8].

Unlike the UCL dataset, the PICS dataset was not used for classifier training. Instead, it served as an independent validation dataset to verify the robustness of the ECG preprocessing methodology prior to multimodal pain classification. Following preprocessing, a total of 26,208 ECG windows were successfully extracted and analyzed.

TABLE I  
PICS DATASET OVERVIEW

| Dataset           | PICS                    |
|-------------------|-------------------------|
| Subjects          | 10 infants              |
| Signal            | ECG                     |
| Sampling Rate     | 500 Hz                  |
| Windows Extracted | 26,208                  |
| Purpose           | ECG pipeline validation |

2) *UCL Dataset (Pain Classification)*: The primary experiments in this study were conducted using the publicly available University College London (UCL) neonatal pain dataset [11] obtained through the UK Data Service. The dataset contains synchronized multimodal physiological recordings acquired during clinically relevant experimental procedures.

A total of 112 neonates were included in the study, resulting in 308 successfully processed recordings out of 310 available recordings. Each recording contains synchronized EEG, ECG, and respiration signals collected during heel lance (pain), sham control, and auditory control procedures.

Pain labels were generated using both experimental condition information and Premature Infant Pain Profile (PIPP) scores. In addition to the complete dataset, a PIPP-confirmed subset was constructed by selecting recordings with clinically significant pain scores ( $\text{PIPP} \geq 7$ ), enabling a stricter evaluation protocol.

TABLE II  
UCL DATASET OVERVIEW

| Dataset              | UCL                    |
|----------------------|------------------------|
| Subjects             | 112 neonates           |
| Processed Recordings | 308 / 310              |
| Signals              | EEG, ECG, Respiration  |
| Sampling Rate        | 2000 Hz (EEG)          |
| Pain Stimulus        | Heel Lance             |
| Control Conditions   | Sham, Auditory         |
| Labels               | Condition + PIPP Score |
| Purpose              | Pain Classification    |

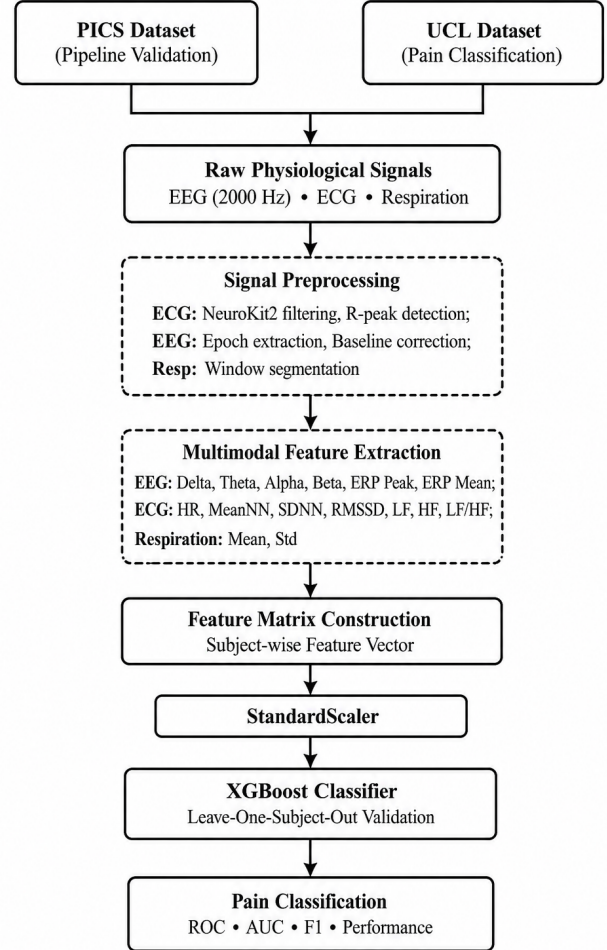


Fig. 1. Overview of the proposed framework.

#### B. Overall Framework

The overall methodology adopted in this work is illustrated in Figure 1. The framework consists of five sequential stages: data acquisition, physiological signal preprocessing, multimodal feature extraction, feature matrix construction, and subject-independent pain classification using XGBoost [12] with Leave-One-Subject-Out (LOSO)-cross-validation [13].

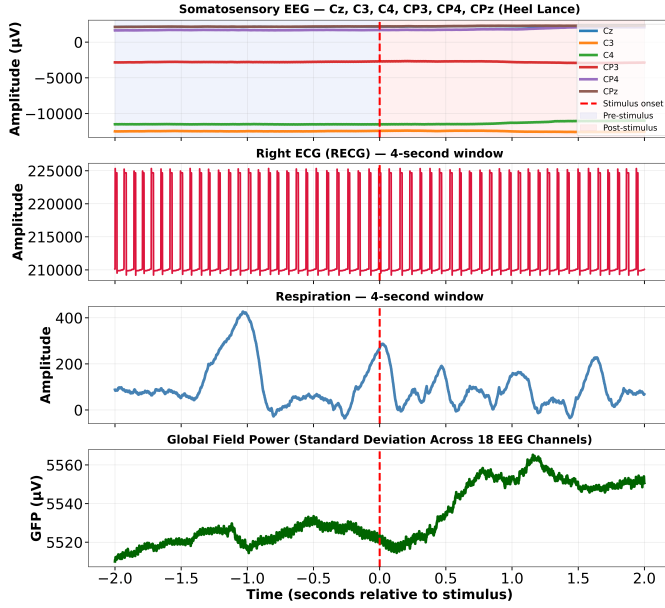


Fig. 2. Representative multimodal physiological recording from the UCL dataset showing synchronized EEG, ECG, and respiration signals aligned to stimulus onset.

### C. Signal Processing

EEG preprocessing involved spectral analysis using Welch’s power spectral density estimation [14], extraction of delta, theta, alpha and beta band power, computation of event-related potential (ERP) characteristics, and estimation of Global Field Power (GFP) [15]. The raw multimodal signals are illustrated in Figure 2

### D. Feature Extraction

Following modality-specific preprocessing, quantitative physiological features were extracted from the EEG, ECG, and respiration signals to construct a unified multimodal feature representation for pain classification. Following feature extraction, a total of 63 features were retained after removing columns with missing values exceeding 30%, yielding a unified feature vector per recording.

For the ECG modality, heart rate (HR) and Heart Rate Variability (HRV) features were computed from the detected R-peaks, including MeanNN, SDNN, RMSSD, pNN50, LF power, HF power, and the LF/HF ratio. EEG features comprised spectral band power (Delta, Theta, Alpha, and Beta) together with event-related potential (ERP) descriptors extracted from the post-stimulus response window. Respiration features were represented using statistical descriptors, including the mean and standard deviation within the analysis window.

Figure 3 presents an exploratory analysis of the ECG-derived physiological features extracted from the PICS dataset. The distributions of HR, RMSSD, and SDNN across individual infants demonstrate both inter-subject variability and consistent physiological trends, while the feature correlation matrix highlights the relationships among the extracted HRV metrics.

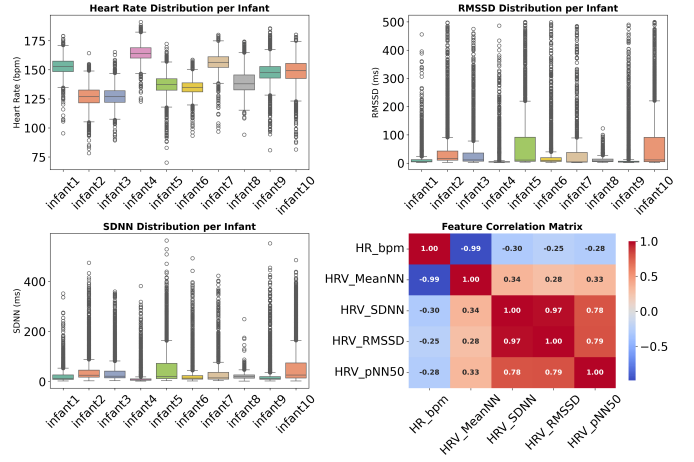


Fig. 3. Exploratory analysis of ECG-derived HRV features extracted from the PICS dataset. Box plots show inter-subject variability in heart rate, RMSSD, and SDNN across 10 infants, alongside the feature correlation matrix.

This analysis provides additional validation of the extracted feature set prior to multimodal classification.

Table III summarizes the complete set of features extracted from each physiological modality and used to construct the final multimodal feature matrix.

### E. Classification

Feature standardization was performed independently within each LOSO fold using StandardScaler to prevent information leakage. The XGBoost classifier was trained with class imbalance compensation via the `scale_pos_weight` parameter, set as the ratio of negative to positive training samples within each LOSO fold. The XGBoost classifier was configured with `n_estimators = 300`, `max_depth = 3`, `learning_rate = 0.03`, `subsample = 0.8`, and `colsample_bytree = 0.8`.

## IV. EXPERIMENTAL RESULTS

Before integrating into multimodal pain classification, the proposed ECG preprocessing and feature extraction pipeline was independently validated using the PICS dataset. The aim of this experiment was to validate the ECG processing methodology before applying it on larger multimodal UCL dataset.

Every recording was filtered, heartbeats located, and HRV features extracted from 60-second windows. Across all ten infants we got 26,208 valid windows, enough to confirm the pipeline behaved consistently across subjects with different recording lengths and heart rates.

Figure 4 shows a representative preprocessing example. The upper panel presents the cleaned ECG signal together with automatically detected R-peaks, while the lower panel shows the corresponding heart-rate profile derived from the detected cardiac cycles. The successful identification of R-peaks in neonatal ECG recordings, despite the presence of motion artefacts and physiological noise, demonstrates the robustness of the proposed preprocessing pipeline. The extracted HRV

TABLE III  
EXTRACTED FEATURES

| Modality    | Features                                                                         |
|-------------|----------------------------------------------------------------------------------|
| EEG         | Delta, Theta, Alpha, Beta Band Power, ERP Peak, ERP Mean, Global Field Power     |
| ECG         | Pre/Post Heart Rate, Heart Rate Change, MeanNN, SDNN, RMSSD, LF, HF, LF/HF Ratio |
| Respiration | Mean, Standard Deviation                                                         |

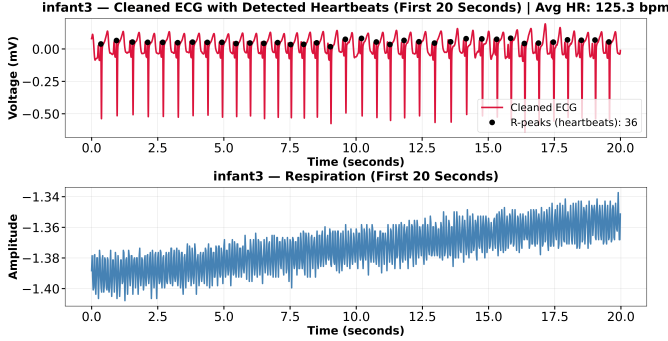


Fig. 4. Validation of the ECG preprocessing pipeline using the PICS dataset. The cleaned ECG waveform with automatically detected R-peaks demonstrates reliable heartbeat detection following signal preprocessing, enabling robust heart rate and heart rate variability (HRV) feature extraction.

features subsequently formed the ECG component of the multimodal feature matrix used for pain classification.

#### A. Validation of the ECG Preprocessing Pipeline Using the PICS Dataset

Before any pain-labelled data was used, the pipeline ran against the PICS recordings as a test. Ten infants across hundreds of hours of continuous monitoring gave a realistic check that the cleaning and peak detection produced sensible results, R-peak detection, and HRV analysis, yielding 26,208 ECG windows. Figure 4 illustrates a representative result: automatically detected R-peaks on cleaned ECG demonstrate reliable heartbeat identification despite motion artefacts, confirming pipeline robustness for neonatal signals.

#### B. Multimodal Pain Classification on the UCL Dataset

The proposed multimodal framework was evaluated on the UCL neonatal pain dataset using Leave-One-Subject-Out (LOSO) cross-validation to ensure subject-independent assessment. EEG, ECG, and respiration features extracted from each recording were combined into a unified feature vector and classified using an XGBoost classifier.

Two evaluation protocols were considered. The first used broad condition-based labels (heel lance versus control), resulting in 53 evaluable subjects. The second used a clinically validated subset containing recordings with available Premature Infant Pain Profile (PIPP) scores, where samples with  $\text{PIPP} \geq 7$  were considered pain events, resulting in 28 subjects.

The quantitative performance under both evaluation protocols is summarized in Table IV.

Figure 5 compares the ROC curves obtained under both evaluation protocols. Both curves remain well above the

TABLE IV  
CLASSIFICATION PERFORMANCE USING LOSO CROSS-VALIDATION

| Evaluation Group      | Subjects | Mean AUC                            | Mean F1-score     |
|-----------------------|----------|-------------------------------------|-------------------|
| Broad Condition-based | 53       | $0.849 \pm 0.285$                   | $0.475 \pm 0.450$ |
| PIPP-confirmed        | 28       | <b><math>0.929 \pm 0.175</math></b> | $0.452 \pm 0.438$ |

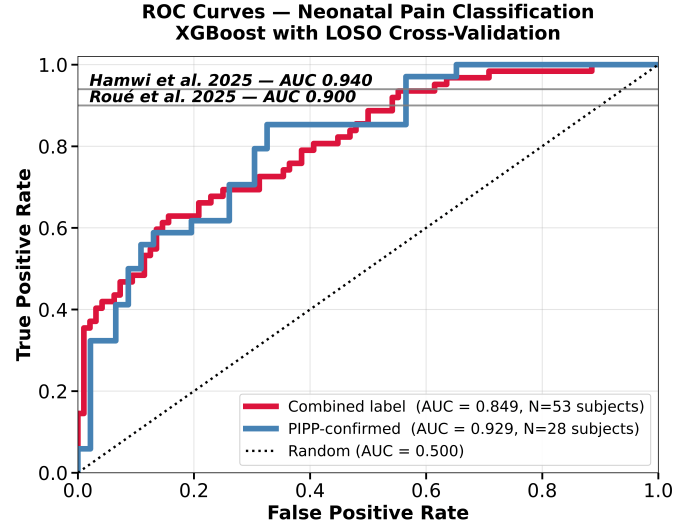


Fig. 5. Receiver Operating Characteristic (ROC) curves obtained using Leave-One-Subject-Out (LOSO) cross-validation for the proposed XGBoost classifier. The framework achieved a mean LOSO AUC of 0.849 using broad condition-based labels (53 subjects) and 0.929 using the clinically validated PIPP-confirmed subset (28 subjects). The diagonal dotted line denotes random classification, while horizontal reference lines indicate previously reported EEG-based performance.

random classification baseline, demonstrating reliable discrimination between pain and non-pain recordings. The PIPP-confirmed cohort achieved a higher mean LOSO AUC (0.929) than the broad condition-based cohort (0.849), indicating that clinically validated pain labels provide more discriminative supervision for multimodal learning.

Overall, the proposed multimodal framework demonstrates that combining cortical (EEG) and autonomic (ECG and respiration) biomarkers enables robust subject-independent neonatal pain classification, with performance comparable to recent state-of-the-art studies while using multimodal physiological information.

#### C. Comparison with Existing Methods

The proposed framework was compared with recent studies that employed the same UCL neonatal pain dataset. Table V summarizes the reported performance of representative machine learning approaches.

TABLE V  
COMPARISON WITH RECENT STUDIES

| Study            | Classifier          | Validation    | AUC               |
|------------------|---------------------|---------------|-------------------|
| Hamwi et al. [5] | Logistic Regression | LOSO          | 0.94 <sup>†</sup> |
| Roué et al. [6]  | Gradient Boosting   | Subject-level | 0.90              |
| Proposed Method  | XGBoost             | LOSO          | <b>0.929</b>      |

<sup>†</sup>Restricted to infants with gestational age  $\geq 34$  weeks.

The proposed framework achieves performance comparable to recently published methods while employing a comprehensive multimodal feature set extracted from EEG, ECG, and respiration signals. Notably, although Hamwi *et al.* [5] reported a higher AUC of 0.94, that result was obtained exclusively on the oldest gestational-age cohort ( $\geq 34$  weeks), where cortical pain responses are most mature and discriminable. The proposed framework achieves an AUC of 0.929 across all gestational-age groups under the same LOSO evaluation protocol, demonstrating that multimodal physiological integration enables robust subject-independent classification without restricting analysis to the most developmentally advanced subgroup. These results shows , the proposed framework gives a reliable and clinically relevant approach for automated neonatal pain assessment.

## V. DISCUSSION

The proposed multimodal framework shows that combining EEG, ECG, and respiration signals provide complementary information for objective neonatal pain assessment. Validation of the ECG preprocessing pipeline on the PICS dataset ensured reliable physiological feature extraction before multimodal model development.

Performance got better from an AUC of 0.849 to 0.929 when we used clinically validated PIPP labels instead of broad condition-based labels, highlighting the importance of accurate ground-truth annotation. Labels which are Clinically validated reduce ambiguity and improve the discriminative ability of the classifier.

The relatively large variation in LOSO performance is expected because each test fold contains only a small number of recordings, which is a known limitation of cross-validation on small clinical datasets [13].

*Limitations:* The evaluable subject count is constrained by PIPP score availability (79 of 112 subjects have scores) and the requirement for mixed-class test sets in LOSO. External validation on an independent neonatal cohort and interpretability analysis using explainable AI techniques remain as future work.

## VI. CONCLUSION

This paper presented a multimodal machine learning framework for automated neonatal pain classification using EEG, ECG, and respiration signals. A two-stage experimental pipeline was employed: the ECG preprocessing and HRV feature extraction workflow was validated on the PICS dataset, and multimodal pain classification was evaluated on the UCL

neonatal pain dataset comprising 308 recordings from 112 neonates using XGBoost with LOSO cross-validation.

Across both evaluation protocols, the classifier reached AUC values of 0.849 and 0.929 — the higher figure coming from the subset of recordings where clinicians had assigned validated PIPP scores. Compared to recent work that used EEG alone, the multimodal approach here held its ground while drawing on a richer picture of what the infant’s body was doing at the moment of the painful stimulus. These results confirm that clinically validated labels and multimodal physiological integration jointly improve subject-independent pain classification.

Future work will investigate model interpretability through explainable AI techniques, extend evaluation to additional neonatal cohorts, and explore real-time deployment for bedside clinical support.

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